

1. Administrative & Study Identification

Full Study Title: Korea Osimertinib Real World Evidence Study to Assess Safety and Efficacy - PLUS (KOREA-PLUS)
Short Title/Acronym: KOREA-PLUS
Protocol Number: D5161R00005
NCT Number: NCT03918304
Sponsor Name: AstraZeneca
Investigational Product: Osimertinib (Tagrisso)
Phase of Development: Phase 4 / Non-interventional (Post-marketing surveillance)
Medical Monitor: AstraZeneca Clinical Operations Lead (Korea)

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess the safety of Tagrisso (osimertinib) for patients with Non-Small Cell Lung Cancer (NSCLC) treated with Tagrisso under the approved indication in Korea.
Secondary Objectives: To assess the efficacy of Tagrisso for patients with NSCLC treated with Tagrisso under the approved indication in Korea.

2.2 Background & Rationale To generate real-world evidence and evaluate the safety and clinical effectiveness of single-agent Tagrisso in a routine clinical practice setting, ensuring it performs according to the approved label in the Korean patient population (regulatory postmarketing surveillance).

3. Study Design & Methodology

3.1 Design Framework Study Type: Observational (Local, prospective, non-interventional, regulatory postmarketing surveillance)
Control Type: Single-arm
Blinding: Open-label
Randomization: N/A

3.2 Planned Enrollment & Duration Target \$N\$: Broad post-marketing population (determined by active prescriptions in participating centers)
Number of Sites: Multicenter (across South Korea)
Estimated Study Start: 11-Jun-2019
Estimated Primary Completion: 27-Oct-2024

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Eligible for, or on active study drug treatment according to the approved prescribing information. 2. Patients must meet one of the following indications: - First-line treatment of patients with locally advanced or metastatic NSCLC whose tumours have epidermal growth factor receptor (EGFR) exon 19 deletions or exon 21 (L858R) substitution mutations. - Treatment of patients with locally advanced or metastatic EGFR T790M mutation-positive NSCLC who have been previously treated with EGFR TKI therapy. - Adjuvant treatment after complete tumour resection in patients with NSCLC whose tumours have EGFR exon 19 deletions or exon 21 (L858R) substitution mutations.

4.2 Exclusion Criteria 1. History of hypersensitivity to the active substance or to any of the excipients of this drug. 2. Pregnancy and/or breast feeding. 3. Current participation in any interventional trial.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Single agent Tagrisso (osimertinib) dosed according to the approved label and at the discretion of the treating physician. **Route of Administration:** Oral. **Duration of Treatment:** Continuous treatment in a real-world setting until disease progression, unacceptable toxicity, or physician/patient decision to withdraw.

5.2 Concomitant & Prohibited Therapy Permitted: Routine concomitant medications allowed per real-world standard of care and local prescribing information. **Prohibited:** Use of other investigational agents (current participation in an interventional trial is excluded).

6. Study Timeline & Visit Schedule

(Note: As a non-interventional observational study, visits align with standard routine clinical practice rather than a strict protocol-mandated schedule.) | Visit ID | Window (Days) | Key Activities/Procedures | | :--- | :--- | :--- | | **Baseline** | Day 0 | Enrollment evaluation, Informed Consent, Initial Prescription & Baseline Characteristics | | **Routine Visits** | Per Standard of Care | Safety monitoring, AE reporting, Real-world efficacy assessments (e.g., standard imaging), Survival tracking | | **End of Study** | Follow-up End | Final safety evaluation, survival status, or treatment discontinuation documentation |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Assessment of the safety of Tagrisso by measuring the incidence and severity of Adverse Events (AEs) and Serious Adverse Events (SAEs). **Measurement Method:** Real-world pharmacovigilance reporting based on routine clinical visits.

7.2 Secondary & Exploratory Endpoints Assessment of real-world efficacy of Tagrisso (e.g., Progression-Free Survival [PFS], Objective Response Rate [ORR], and Overall Survival [OS] based on standard-of-care clinical evaluations).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Spontaneous reporting by healthcare professionals during routine consultations. **Serious Adverse Events (SAEs):** Reported to AstraZeneca pharmacovigilance teams and local regulatory authorities according to post-marketing requirements. **Data Monitoring Committee (DMC):** Not typically required for post-marketing observational studies, but safety signals are routinely monitored by the sponsor.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Analysis Set (all patients who received at least one dose of Tagrisso and had safety follow-up data). **Sample Size Justification:** Based on the regulatory requirements for post-marketing surveillance in Korea. **Handling of Missing Data:** Handled per standard epidemiological practices for observational data (e.g., observed cases, censoring rules for time-to-event endpoints).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study is conducted in accordance with Good Pharmacoepidemiology Practices (GPP) and local post-marketing surveillance regulatory requirements in Korea. **Monitoring Plan:** Pharmacovigilance data collection via standard regulatory portals. **Audit Trail:** Source data verification and documentation mapping back to hospital EMRs per local laws.

11. Study Identifiers & Governance

Primary ID: D5161R00005 **Registry Number:** NCT03918304 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** KOREA-PLUS **Official Regulatory Title:** Korea Osimertinib Real world Evidence study to Assess safety and efficacy - PLUS

12. Clinical Framework (NSCLC Specific)

Primary Condition: Non-Small Cell Lung Cancer (NSCLC) **Diagnosis Threshold:** EGFR exon 19 deletions, exon 21 (L858R) mutations, or T790M mutation-positive **Medical Methodology:** Local Real-World Evidence (RWE) / Regulatory Postmarketing Surveillance **Optimization Target:** Real-world safety and efficacy characterization

13. Study Procedures & Methodology

Management Pathway: Standard local clinical pathway for advanced/metastatic NSCLC **Education Protocol (HCP):** Local approved prescribing information guidelines **Education Protocol (Patient):** Standard of care counseling provided by the treating oncologist **Quality Audit Mechanism:** Routine regulatory pharmacovigilance tracking

14. Observation & Follow-Up Schedule

Total Duration: Approximately 5.3 years (June 2019 to October 2024) **Onsite Visit Cadence:** According to routine clinical practice **Remote Monitoring Frequency:** Pharmacovigilance tracking **Checklist Review Interval:** Standard periodic safety update report (PSUR) intervals

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				